

6.1 Suspected Abuse of Residents and Team Members

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens enforces a zero-tolerance policy for abuse of or by residents or team members. All team members are obligated by law when there is a reasonable basis to report abuse, neglect, suspected abuse or inappropriate behavior without fear of retaliation from the General Manager of the Community or from Merrill Gardens. Residents have the same rights and are encouraged to exercise them. Merrill Gardens seeks to ensure that abusive situations do not occur. In the event an incident occurs, management will act quickly and fairly to investigate and to take decisive action as appropriate.

Definitions:

Abuse

An act by any individual which injures, exploits or in any way jeopardizes a person's health, welfare, or safety, including but not limited to:

- Physically damaging or potentially damaging non-accidental acts
- Emotionally damaging verbal behavior and harassment
- Sexual misconduct, including exploitation and mistreatment through inappropriate touching, inappropriate remarks, or encouraging participation in pornography or prostitution
- Visual misconduct, including drawings, cartoons or gestures

Exploitation

The illegal or improper use of a resident's resources, labor, or services for another person's profit or advantage.

Neglect

Failure to provide the goods or services that a reasonable person would provide that presents a clear and present danger to the health, welfare or safety of the person.

FL: Adverse Incident: *An event over which community personnel could exercise control rather than as a result of the resident's condition and results in: death, brain or spinal damage, permanent disfigurement, fracture or dislocation of bones or joints, any condition that required medical attention to which the resident has not given his or her consent (including failure to honor advanced directives), any condition that requires the transfer of the resident from the community to a unit providing more acute care due to the incident, an event that is reported to law enforcement or it's personnel for investigation, resident elopement (if the elopement places the resident at risk of harm or injury).*

NV: Isolation: Willfully, maliciously and intentionally preventing a resident from having contact with another person.

SC: Physical Abuse: The act of intentionally inflicting or allowing to be inflicted physical injury on a resident by an act or failure to act. Physical abuse includes, but is not limited to, slapping, hitting, kicking, biting, choking, pinching, burning, actual or attempted sexual battery, use of medication outside the standards of reasonable medical practice for the purpose of controlling behavior, and unreasonable confinement. Physical abuse also includes the use of a restrictive or physically intrusive procedure to control behavior for the purpose of punishment except that a therapeutic procedure prescribed by a licensed physician or other legally authorized healthcare professional or that is part of a written ICP by a physician or other legally authorized healthcare professional is not considered physical abuse. Physical abuse does not include altercations or acts of assault between residents.

SC: Psychological Abuse: The deliberate use of any oral, written, or gestured language or depiction that includes disparaging or derogatory terms to a resident or within the resident's hearing distance, regardless of the resident's age, ability to comprehend, or disability, including threats or harassment or other forms of intimidating behavior causing fear, humiliation, degradation, agitation, confusion, or other forms of serious emotional distress.

WA: Abandonment: Action or inaction by a person with a duty of care that leaves the resident without the means or ability to obtain necessary food, clothing, shelter or health care.

Guidelines

Abuse of Resident

- All team members are mandatory reporters of abuse. Anyone suspecting abuse of a resident must report any form of abuse immediately. State specific requirements must be followed.

CA: All new team members and volunteers will sign a SOC341A at hire or before beginning duties. The signed form will be kept in the personnel files.

- Report any suspected or observed abuse of a resident(s) immediately to the General Manager or supervisor of the community.
- All reports of suspected or alleged abuse, exploitation, or neglect will be investigated by the General Manager or designee. The General Manager or designee will document the investigation, investigative actions and the outcome.

AL: A detailed and accurate report is to be completed within 24 hours of the incident and provided to the General Manager for review. The investigation file is to be available for inspection & copying by ADPH upon request.

AZ: The General Manager will initiate an investigation and document the following within 5 working days of the initial report to APS:

- The suspected abuse, neglect, or exploitation, including the dates, times and descriptions of the abuse, neglect, or exploitation;
- Any action taken to stop the suspected abuse, neglect, or exploitation;

- *Reports made to law enforcement, Adult Protective Services, and the Arizona Department of Health Services;*
- *A description of any injury to the resident related to the suspected abuse or neglect and any changes to the resident's physical, cognitive, functional, or emotional condition;*
- *The names of witnesses to the suspected abuse, neglect, or exploitation; and*
- *The actions taken by the General Manager to prevent the suspected abuse, neglect, or exploitation from occurring in the future.*
- *Documentation shall be maintained for at least 12 months.*
- Community team members will make every attempt to ensure the safety and welfare of the resident(s) for the duration of the investigation of suspected abuse.
- If it is suspected that the General Manager may be involved in the incident or in its cover-up, then team members are directed to contact the Vice President of Operations immediately. Documentation of a thorough investigation will be made. An incident report should be completed.
- Team members and/or residents who report suspected abuse may do so without fear of retaliation from the General Manager or from Merrill Gardens.
- Merrill Gardens will assist with arrangements for a counselor or other professional knowledgeable in the field of rape and sexual assault to question or interview the resident, and provide counseling or intervention.

Protection of Resident Following Reported Abuse

- Team members witnessing or reporting abuse shall take every measure to separate the resident from the suspected abuser.
- Qualified team members will perform First Aid measures if appropriate.
- 911 will be called as appropriate.
- Notification of the General Manager is mandatory for any incidence of abuse or suspected abuse.
- The General Manager will contact appropriate resident family member(s) and the Vice President of Operations.
- The General Manager will call the State agency hotline to report any allegation of abuse as required by state.

AL: Reporting Requirements: *A report is to be made to the ADPH online reporting system within 24 hours of the incident & should include: (1) Community name & phone number; (2) Time & date of the report; (3) Reporter's name; (4) Name of resident(s), team member(s) or visitor(s) involved in the incident; (5) Names of team members on duty at the time of the incident; (6) Date & time of the incident; (7) Any injury to resident(s); (8) Action taken by the community in response to the incident.*

AZ: Reporting Requirements: Any mandatory reporter who has a reasonable basis to believe abuse, neglect, or exploitation has occurred must immediately report the incident to Adult Protective Services and to law enforcement. The initial APS report should be made using the online reporting system if practical. The report must include at least: (1) the names and addresses of the adult and any persons having custody or control of the adult, if known; (2) the adult's age and nature and extent of the adult's vulnerability; (3) the nature and extent of the adult's injuries or physical neglect or of the exploitation of the adult's property; and (4) any other information that the person reporting believes might be helpful in establishing the cause of the adult's injuries or physical neglect or of the exploitation of the adult's property. The initial report to APS must be followed up by a written report within 48 hours or the next business day if the 48 hours expires on a weekend or holiday.

CA: Reporting Requirements: Any mandated reporter who, in their professional capacity, or within the scope of their employment has observed, suspects or has knowledge of an incident (regardless of whether it occurred at the community or off-site), that reasonably appears to be physical abuse (including sexual abuse), abandonment, isolation, financial abuse, abduction, or neglect (including self-neglect), or is told by a resident that they have experienced behavior constituting abuse as listed above, must report suspected abuse by telephone immediately or as soon as practically possible and by written report (SOC 341) sent within two (2) working days to the appropriate agency as follows:

- Suspected abuse occurring at the community: Mandated reporters must report to the Long-Term Care Ombudsman or local law enforcement agency.
- Suspected abuse occurring offsite: Mandated reporters must report to the adult protective services agency or local law enforcement.

A written report (SOC 341) will also be submitted to the local licensing office and to the person responsible for the resident within 7 days of the occurrence of any incident which threatens the welfare, safety or health of a resident, such as physical or psychological abuse of a resident by staff or other residents. This report will include the resident's name, age, sex and move-in date; date & nature of event; attending physician's name, findings, and treatment, if any; and disposition of the case.

FL: Reporting Requirements: Everyone, including professionally mandatory reporters, should contact the FL Abuse Hotline when they know or have reasonable cause to suspect that a vulnerable adult has been abused, abandoned, neglected, or exploited.

A preliminary report shall be made within **1 business day** after the occurrence of an adverse incident.

A full report shall be made within **15 days** of the adverse incident and must include the community investigation.

- How to make a report:
Web Reporting FL Agency for Health Care Administration online at
<https://weblics.fdhc.state.fl.us/ahca/>

*Information needed to report: Specific descriptions of the incident(s) or the circumstances contributing to the risk of harm; including **who** was involved, what occurred, **when** and **where** it occurred, **why** it happened (if known), the extent of any **injuries** sustained, and what the victim(s) said happened; and any other pertinent information.*

- *For emergency situations or if the victim is in imminent danger, call 911 and follow up with a call to the Abuse Hotline 1-800-96-ABUSE.*

*A **monthly report** of any liability claim filed against the community will be submitted to the FL Agency for Health Care Administration online at <https://weblics.fdhc.state.fl.us/ahca/>. The report must include the name of the resident, the dates of the incident leading to the claim, if applicable, and the type of injury or violation of rights alleged to have occurred. This report is not discoverable. If no liability claim has been filed in a given month, a report is not required.*

GA: *Reporting requirements: The General Manager or designee shall report to the Division of Healthcare Facility Regulation whenever abuse of a resident is suspected or alleged. Refer to GA Reporting Requirements.*

NV: *Reporting Requirements: A report is to be made as soon as reasonably practical, but not later than 24 hours after there is knowledge of or reasonable cause to believe abuse, neglect, exploitation or isolation of a resident has occurred. A report is to be made to (1) The local office of DHHS; (2) law enforcement; (3) County APS; OR (4) A toll-free telephone service designated by DHHS (702) 486-3587. The report may be faxed to (702) 486-6520.*

SC: *The administrator or his or her designee shall report abuse and suspected abuse, neglect, or exploitation of residents to the South Carolina Long-Term Care Ombudsman Program in accordance with 1976 Code Section 43-35-25.*

WA: *Reporting requirements: (1) When there is reasonable cause to believe that abandonment, abuse, financial exploitation, or neglect of a vulnerable adult has occurred, mandated reporters shall immediately report to the department.*

(2) When there is reason to suspect that sexual assault has occurred, mandated reporters shall immediately report to the appropriate law enforcement agency and to the department.

(3) When there is reason to suspect that physical assault has occurred or there is reasonable cause to believe that an act has caused fear of imminent harm:

(a) Mandated reporters shall immediately report to the department (800) 562-6078; and

(b) Mandated reporters shall immediately report to the appropriate law enforcement agency, except as provided in subsection (4) of this section.

(4) A mandated reporter is not required to report to a law enforcement agency, unless requested by the injured vulnerable adult or his or her legal representative or family member, an incident of physical assault between vulnerable adults that causes minor bodily injury and does not require more than basic first aid, unless:

(a) The injury appears on the back, face, head, neck, chest, breasts, groin, inner thigh, buttock, genital, or anal area;

(b) There is a fracture;

(c) There is a pattern of physical assault between the same vulnerable adults or involving the same vulnerable adults; or

(d) There is an attempt to choke a vulnerable adult.

(5) When there is reason to suspect that the death of a vulnerable adult was caused by abuse, neglect, or abandonment by another person, mandated reporters shall, pursuant to RCW 68.50.020, report the death to the medical examiner or coroner having jurisdiction, as well as the department and local law enforcement, in the most expeditious manner possible.

Sexual Abuse

If sexual abuse is suspected:

- Arrange for an immediate medical examination of the victim, with prior notice to the examining physician that the patient may have been raped or sexually abused;
- Arrange for a counselor or other professional knowledgeable in the field of rape and sexual assault to question or interview the resident, and to provide counseling or intervention, when appropriate.
- Only team members with special training in the field of rape and sexual assault will question the victim or the suspected perpetrator regarding the alleged incident, unless the State licensing agency, the Police, or the Prosecutor's Office asks otherwise.

Abuse of Team Member

All team members have the obligation to protect residents from abuse, and to report any incidents or suspected incidents when there is a reasonable basis to believe abuse occurred. At the same time, team members are not obliged to endure abuse of any kind themselves, whether at the hands of other team members or residents. This information can be reported to a supervisor, General Manager, Team member support line or the Corporate office.

Report such incidents immediately to the General Manager of the Community, or, as in the case of suspected resident abuse, to the Vice President of Operations if the Community General Manager is involved or suspected in the abuse.

Training

All team members will receive training in residents' rights and abuse reporting. Team members will receive ongoing training through staff in-services specifically addressing residents' rights and employee responsibilities for reporting abuse, in addition to initial training at orientation.

GA: Training on resident's rights and the Long-term Care Abuse Reporting Act will be completed within the first week of employment. Each team member will be provided a copy of both Residents' Rights and Rules and Regulations for Assisted Living Communities Care Homes) as detailed CHAPTER 111-8-63.25 RULES AND REGULATIONS FOR ASSISTED LIVING and have access to them in this policy manual, and review Article 4, Reporting Abuse or Exploitation of Residents in Long-term Care Facilities (O.C.G.A. §31-8-50 et. seq.) and will acknowledge receipt by signing and dating the last page of the documents. A copy of each signed document will be maintained in the employee's personnel file. Team members will receive continuing training through in-services specifically addressing residents' rights and employee responsibilities under the Long-Term Care Abuse Reporting Act in addition to initial training at orientation.

Merrill Gardens Resident Incident Form

Community:

Reported By:

Selected Party (Resident Name):

Incident Date:

Unit #:

Incident Time:

Care Level (IL/AL/GH):

Reported Date:

INCIDENT TYPE (check one)

- ☐ Fall - No Injury
- ☐ Fall-Injury-First Aid
- ☐ Fall - Injury-Medical Transport
- ☐ Fall-Fracture
- ☐ Missing Resident - AL
- ☐ Missing Resident - GH
- ☐ Medication-No adverse effect
- ☐ Medication - Adverse Effect
- ☐ Skin Issue - Bruising/Discoloration

- ☐ Skin Issue - Skin Tear/Laceration
- ☐ Skin Issue - Ulcer
- ☐ Skin Issue-Burn
- ☐ Skin Issue - Other
- ☐ Suicide-Ideation/Attempted/Suicide
- ☐ Death - Expected
- ☐ Death - Unexpected
- ☐ Theft - Medication
- ☐ Theft - Property

- ☐ Medical - First Aid
- ☐ Medical - 911
- ☐ Medical - Other
- ☐ Abuse Allegation - Resident to Resident
- ☐ Abuse Allegation- Staff to Resident
- ☐ Abuse Allegation- Other
- ☐ Visitor Incident

DESCRIPTION

Description:

LOCATION

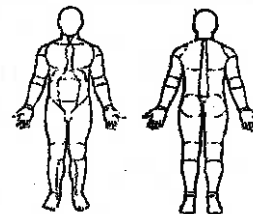
Location of Incident:

Notes:

INJURY / CONCERN

Describe Injury:

Circle Location of Injury/Concern:



VITAL SIGNS

Date / Time Vitals Taken:

Vitals Taken by:

Pulse: _____ Breathing Rate: _____ Temperature: _____ Blood Pressure: _____

WITNESS

Witness Name(s)

Notes

NOTIFICATION LOG**(Must notify responsible party and health care provider (n/a for WA IL))**

Date - Time	Notification Method	Person Contacted	Notified By	Notes
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MG ADDITIONAL INFORMATION (circle applicable responses)**Activity Prior:**

- | | | | |
|---|--|---|--|
| ☐ Toileting | ☐ Making Bed | ☐ Transfer to/from chair | ☐ Other (specify) |
| ☐ Shower/Bathing | ☐ Rolled out of Bed | ☐ Transfer to/from scooter | ☐ Unknown |
| ☐ Dressing | ☐ Getting in/out of bed | ☐ Walker | |

Mental Status Prior to Incident:

- | | | | |
|---|--|--|--|
| ☐ Usual | ☐ Restless/Agitated | ☐ Verbally Aggressive | ☐ Other (specify) |
| ☐ Confused / Disoriented | ☐ Hallucinating | ☐ Combative | ☐ Unknown |

Assistive Device: ☐ Wheelchair ☐ Walker ☐ Cane ☐ Scooter ☐ Other ☐ N/A

Use of Assistive Device: ☐ Used ☐ Has but not used ☐ Unknown ☐ N/A

Environmental – Floor: ☐ Wet ☐ Dry ☐ Unknown ☐ N/A

Environment – Type of Surface: ☐ Carpet ☐ Throw Rug ☐ Linoleum/Tile ☐ Unknown ☐ Other ☐ N/A

Environmental – Level: ☐ Flat ☐ Incline ☐ Transition between surfaces ☐ Unknown ☐ Other ☐ N/A

Environmental – Lighting: ☐ On ☐ Off ☐ Unknown ☐ N/A
Environmental – Low or Elevated Furniture Heights:

- ☐
- Sofa
- ☐
- Chair
- ☐
- Bed
- ☐
- Toilet Seat
- ☐
- Shower Seat
- ☐
- N/A

Environmental – Footwear: ☐ Non-skid Shoes ☐ Smooth Soles ☐ Socks / Nylons ☐ Barefoot ☐ Unknown ☐ Other
Appearance of Resident when Found:

- | | | |
|---|---|--|
| ☐ Standing | ☐ Confused / Disoriented | ☐ Short of Breath |
| ☐ Sitting | ☐ Agitated / Combative | ☐ Complaint of Pain |
| ☐ Laying on Back | ☐ Weepy | ☐ Bumped Head |
| ☐ Laying on Stomach | ☐ Non-Responsive | ☐ Other (specify) |
| ☐ Laying on Right Side | ☐ Bleeding | |
| ☐ Laying on Left Side | ☐ Headache or Dizziness | |
| ☐ Oriented | ☐ Slurred Speech | |

Was Resident Able to Get Up Independently? ☐ Yes ☐ No ☐ Unknown

MG NEXT STEPS TAKEN

Was 911 or Ambulance called?

Time Called:

Was person involved taken to ER?

Name of hospital if transported:

Transported by:

Was person admitted to hospital?

Name(s) of person responding to incident:

Signature of person preparing report:

(Signature)

(Position)

(Date)

Follow Up Actions / Steps if Applicable:

(sections below to be completed by RCD/HSD/GHD, circle applicable responses)

MG AP INVESTIGATION

Medication changed in past 2 weeks?

☐ Yes ☐ No ☐ Unknown

Is resident on medications which contribute to bruising?

☐ Blood Thinner ☐ Anti-inflammatory ☐ Steroid ☐ None ☐ Unknown

Is resident on medications which impair alertness or balance?

☐ Sleep aids ☐ Psychoactives ☐ Allergy/Cold/Flu ☐ None ☐ Other

Is there a change to resident's habits?

☐ Sleep Patterns ☐ Urinary Habits/Toileting Needs ☐ None ☐ Poor Appetite/Refusing to Eat
☐ Refusing Medication ☐ Refusing ADL Assistance ☐ Unknown

MG AP CONCLUSION & RECOMMENDATIONS

Abuse Ruled Out (indicate basis for conclusion)?

Recommendations:

- | | |
|--|---|
| ☐ Progress Note Updated (n/a for WA IL) | ☐ NSA Initiated |
| ☐ Alert Charting Initiated | ☐ NSA Updated |
| ☐ PT/OT Evaluation Requested | ☐ MMSE Updated |
| ☐ Medication Review Requested | ☐ Self-Med Eval Updated |
| ☐ UA Requested | ☐ Outside Care Provider-Coordinate of Plan Updated |
| ☐ Doctor's Appointment Requested | ☐ Family Care Conference Scheduled |
| ☐ Fax / Call MD | ☐ Other |

SIGNATURES

(RCD/HSD/GHD Signature)

(Date)

(General Manager Signature)

(Date)

6.2 Falls Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

A pattern of unsteadiness and falls may indicate an emerging health concern for the resident. For this reason, and to help ensure the safety of our residents, all falls must be reported to the Physician and responsible party, and a documented investigation completed. All residents must be evaluated for risk for falls, and steps taken to reduce the risk. The goal is to work with the care team and the resident/family to address the fall risk with appropriate approaches early on. Approaches need to be manageable for the resident/family and the care team to be effective.

Guidelines

What to Do If A Resident Falls

If a resident falls, remember that he or she may be embarrassed and may say everything is okay in order to take the attention away from the situation. Also remember that some residents may not be as sensitive to pain as would normally be associated with an injury. Treat the resident with respect when helping them in these situations but keep focused on discovering the cause and extent of the injury.

Any time a resident bumps their head, or reports that they bumped their head, as soon as a team member becomes aware of it (including falls that occur off-site or are reported up to a week later) the resident should be asked to see a medical provider for evaluation within 24 hours or as soon as possible.

911 should be called for medical evaluation if the resident has a visible injury to the head or complains of headache or vision problems or is not acting as they normally do within 24 hours of the fall. If the resident's emergency contact is present with the resident and the resident prefers to have them take them immediately to be medically evaluated, this would be acceptable in place of calling 911. We will not wait for the emergency contact to arrive if they are not already at the community.

The head includes any part of the body above the neck. Even if no head injury is apparent, there may be a serious injury requiring medical attention.

Suspected Serious Injury

Do not assess the seriousness of the injury yourself. Solicit information from the resident about what happened and his/or her condition. Call 911 for assistance if:

- The fall involves any head or neck injury.
- The resident cannot get up by (him) herself with assistance, or

- The resident experiences pain in attempt to move.

Appears to Be No Injury

If the resident thinks he/she can get up, do the following:

1. Place a chair or other supporting device within the resident's reach.
2. Stabilize the chair or device while the resident attempts to stand.
3. You may assist the resident to stand, but do not lift them if they cannot bear weight.
4. If the resident experiences pain in trying to get up, call 911.

Follow Up

- Document and file an Incident Report for all falls, even minor falls, though they may not constitute an immediate issue. For Assisted Living residents, complete an incident report, make an entry in the Progress Notes, and reference in the communication Log.
- Be supportive of the resident: After any fall, however minor, a resident may feel vulnerable for a period of time. Periodically check with the resident for reassurance and to promote safety. These checks should be documented (usually on the Service Log).
- If a pattern of unsteadiness and falls begins to emerge, the family and the physician should become involved immediately.
- Update the Service Plan with the date of the fall. If the fall indicates a change of condition, review and/or update the Service Plan including fall prevention approaches as appropriate. Complete a Progress Note or document on the incident report indicating whether the fall indicates a change of condition, and why it does or doesn't appear to be a result of abuse or neglect.

Fall Prevention

Prevention of falls and minimizing risk of injuries are important to all residents. The following are suggestions for minimizing the risk of falls for all residents:

- Minimize clutter in the resident apartment to reduce the chance of tripping.
- Pathways to the bathroom should be kept clear.
- Avoid the use of area rugs.
- Avoid the use of extension cords or placing cords across walking paths.
- Use of a cordless phone.
- Use of handrails when walking in the halls & on stairs.
- Footwear should have non-skid soles, fit and provide support.
- Sitting on the edge of the bed for a moment before standing.
- Limiting what is carried, so that balance and visibility are not impaired.
- Being aware of physical limitations that might contribute to falls.

Fall Risk Evaluation and Service Planning

Fall Risk Evaluation

All residents will be evaluated at move-in to identify residents who may be at risk for falls. Residents identified at risk for falls should have interventions put in place to decrease the risk.

Negotiated Service Agreement

Residents identified as at risk for falls will be asked to participate in and sign a negotiated service agreement (NSA). This agreement documents notification of the risk of falling, and interventions to decrease the risk. This agreement becomes part of and should be kept in the resident care chart with the Service Plan.

Sample Interventions

The following potential interventions are provided as a resource for developing the individualized approach for residents identified at risk for falls. The list is not all inclusive, and not all approaches are appropriate for all residents.

Transfer assistance	Motion detector light in apartment
Ambulation assistance	Night light in apartment
Eye glasses within reach	Toileting reminder/assist at bedtime
Lowered bed	Toileting reminders/assist during day
Cane or walker	Evaluate position of bed
Wheelchair for distances	Evaluate environment for hazards
Non-skid footwear on when ambulating	Request rehab consult
Drug regimen review	Dietary consult
Pain medication	Involve in supervised activities
Emergency pendant	Mobility alarm (check state regulations)

Physician Notification of Resident Condition

This form is faxed or scanned to the primary physician to notify them of a resident fall in the community. This form should also be used when the resident has been identified at risk for falling. Documentation of successful transmission is to be placed in the record until the physician response is returned. When the form is returned with a response from the physician, the recommendations must be implemented, and the form filed in the chart. A phone call should be placed to the physician if the form has not been returned in a few days.

Fall History & Actions Recommended

If a resident falls or continues to fall even with preventive approaches, review those approaches and modify them where appropriate. In some cases, we may need to include additional support to supplement our services. That may include: PT, adaptive equipment, home health, sitters, etc. The nature of the falls should be reviewed. We should consider whether the resident is appropriate for our community with the services and support available. Before any move-out notice is issued, there should be a review with the Vice President of Operations to determine whether we can meet the needs of the resident.



Fact Sheet-Calling 911 When a Resident Bumps Their Head

- Any time a resident bumps their head, or reports that they bumped their head, as soon as a team member becomes aware of it (including falls that occur off-site or are reported up to a week later) the resident should be asked to see a medical provider for evaluation within 24 hours or as soon as possible.
- 911 must be called for medical evaluation if the resident has a visible injury to the head or complains of headache or vision problems, or is not acting as they normally do within 24 hours of the fall. If the resident's emergency contact is present with the resident and the resident prefers to have them take them immediately to be evaluated medically, this would be acceptable in place of calling 911. We will not wait for the emergency contact to arrive if they are not already at the community.
- A life-threatening head injury may have no outward symptoms. There are a multitude of tiny blood vessels in the brain. As we age, the walls of these vessels are much less elastic. Just like the skin of our residents, they can tear very easily. A bump can cause a rupture in one of these blood vessels that can bleed in to the brain cavity in the skull causing pressure on the brain. What this can do is cause a pressure area that damages the brain and causes loss of function. If the leak is large, it is called an acute subdural hematoma, which can cause death fairly quickly-hours or days. If untreated, a slow leak, called a chronic subdural hematoma can cause loss of function and may lead to death. This can occur over a period of days, weeks or longer.
- Symptoms of a subdural hematoma: Recent head injury, loss of consciousness after head injury, headache, weakness, numbness, can't speak, slurred speech, nausea & vomiting, lethargy, seizures. Remember, there may not be obvious symptoms.
- A subdural hematoma is an emergency medical condition. Medications such as diuretics to reduce swelling, anticonvulsants to control or prevent seizures, or even emergency surgery to relieve pressure may be needed.
- A bump to any part of the head should have medical evaluation. This includes the eye, nose, cheek, chin, ear or any other part of the body above the neck. Think back to the subdural hematoma.
- A resident or their family member cannot refuse to have us call 911. We will call. The resident can refuse to be treated or transported. The EMT will most likely have them sign a release form in this case.
- We ask that we be provided a copy of any medical evaluation or release.



Negotiated Service Agreement

Resident: _____

This agreement documents the agreement between the Community and _____ [name of family representative] regarding the care of the above-named resident. This agreement provides formal notification that the resident listed above:

- ☐ Has been determined to be at risk for falls
- ☐ Has been determined to be at risk for elopement
- ☐ Has been determined to be at risk for skin breakdown
- ☐ Has actual skin breakdown
- ☐ Other: _____

Your family member has been evaluated by the Community and is at risk for injury, which may cause permanent disability or be life threatening. The following interventions/practices are recommended to enhance the safety of your family member: [Indicate preference by checking box and initialing]

- _____
☐accepted ☐declined _____
- _____
☐accepted ☐declined _____
- _____
☐accepted ☐declined _____
- _____
☐accepted ☐declined _____

I have been given the opportunity to discuss my family member's condition and these interventions with the Community staff. I understand that these injuries may have severe consequences, and that the intervention(s) listed above may lessen the risk for an injury. However, I understand that the community and Merrill Gardens can make no guarantee that my family member will not injure themselves. I understand that assisted living programs are specifically designed to offer residents more independence and autonomy, and that 24-hour care and supervision is not provided on an individual basis. I acknowledge and agree that these interventions are not without risk and are impacted by the choices my family member makes.

Resident: _____ Date: _____

Family/Responsible Party: _____ Date: _____

Community Representative: _____ Title: _____

6.3 Missing ResidentRevised 10/24/2018 Reviewed 10/24/2018

Policy Purpose

Seniors may develop memory-related problems as a consequence of the natural aging process, or as a result of Alzheimer's Disease or related dementia. Current health issues, such as a urinary tract infection may also contribute to confusion. Residents exhibiting these conditions may have a tendency to wander away from the Community. The following procedures help protect the resident and limit the risk of such incidents occurring. The General Manager or designee is responsible for implementing each part of the response process and may delegate tasks to appropriate team members.

Definitions

Missing resident

A resident who is not able to be located in the community or on the premises and has not signed out.

Elopement

A cognitively impaired resident who should not leave the community unsupervised according to their service plan, who leaves the community or a secured area without supervision or the knowledge of staff.

Guidelines

An identification photo of each resident at risk for elopement will be available to all team members and law enforcement as needed at the reception desk or assisted living office.

Responding to a Door Alarm in the Garden House

If a door alarm sounds and it is not determined who set off the alarm, a head count of the residents is to be made. A quick search of the area surrounding the Garden House should be made if all residents are not accounted for to attempt to locate a resident before they leave the immediate area.

Search Efforts

If a resident is missing at any time, a rapid and serious search is conducted of all common areas, the perimeter of the building, and the resident's apartment.

1. Check the resident sign out register to verify the resident did not just go out.
2. Check the resident's records for notes on tendencies to wander away from the premises.
3. Search the community: Resident apartment, common areas, friend's apartments, and community grounds.
4. Check with the resident's friends & relatives whom he/she may be visiting.

5. If the resident is not found, immediately notify the General Manager, who will call the responsible party and or any identified legal representative.
6. Call 911 for police involvement in the search. Seconds may save a life. Act quickly and decisively to locate a missing resident.

GA: *A team member will call the local police department to report the elopement of any resident from the community within 30 minutes of the staff receiving actual knowledge of the person is missing from the community in accordance with the Mattie's Call Act.*

SC: *The administrator or his or her designee shall report every incident involving a resident that leaves the premises for more than twenty-four (24) hours without notice to staff members of intent to leave to local law enforcement, the resident's responsible party, and the Department. The administrator or his or her designee shall immediately notify local law enforcement and the responsible party by telephone when a cognitively impaired resident leaves the premises for any amount of time without notice to staff members.*

7. Notify VPO and Public Relations for further direction.

GA: *The General Manager or designee will notify The Georgia Division of Healthcare Facility Regulation (DHR). The community will report the initiation and discontinuation of a Mattie's call to DHR within thirty (30) minutes of communications with local law enforcement authorities having occurred. After the facility has notified local law enforcement that a resident is missing or has been found, the facility must notify DHR by:*

- (a) *Faxing the incident report to DHR's regular complaint/incident intake fax number at 404-657-5731; or*
- (b) *Submitting the incident report over the Internet website for complaints indicating in the description of the incident section that it is a facility self-report at <https://services.georgia.gov/dhr/reportfiling/>; or*
- (c) *Calling the complaint intake phone numbers at 404-657-5726, 404-657-5728 or 1-800-878-6442. If voicemail is heard, leave a detailed message describing the incident or event, the community's response to ensure the safety of the other residents in care, the name of the reporter, the phone number for follow-up, date and time.*

Documentation

Document all incidents. This documentation helps provide a pattern of behavior for evaluation and assists in making decisions about the resident's ongoing care. If a pattern of such behavior becomes apparent, and the resident is clearly in need of a more secure environment, then he or she may have to be relocated. In any case, documentation is very important, as is the involvement of the General Manager, and, in some cases, the state licensing agencies.

GA: *Documentation that the Georgia Division of Healthcare Facility Regulation was notified must be completed and retained.*

Elopement Drills

The elopement drill has been developed to train and drill staff on the appropriate policy and procedures for searching and reporting a missing resident. Merrill Gardens' greatest exposure in these events is on

the second and third shifts and on weekends when members of the Management Team may not be present. Below is the outline for the protocol for elopement drills.

- Elopement drills are to be held monthly (quarterly on each shift).
- GM will direct, and Key Staff Members will monitor each drill.
- Each drill will be documented on the Elopement Drill Form.
- Training will be conducted for all staff on appropriate policies and procedures to follow.
- Elopement Drill policy and procedure will be reviewed during new hire orientation.



Elopement Drill Report

Elopement drills should be conducted at least quarterly for each shift.
Drills should be completed in accordance with state requirements, if applicable.

Community Name		
Date	Time _____ ☐ a.m. ☐ p.m.	Shift ☐ Day ☐ Evening ☐ Night
SIMULATED ELOPEMENT DRILL		
Brief Description of simulated elopement		
Type of notification initiated		
GM notified of elopement?	☐ Yes	☐ No ☐ N/A
VPO notified of elopement (simulated)?	☐ Yes	☐ No ☐ N/A
911 call made (simulated)?	☐ Yes	☐ No ☐ N/A
Responsible party notified of elopement (simulated)?	☐ Yes	☐ No ☐ N/A
Resident's apartment searched?	☐ Yes	☐ No ☐ N/A
Sign out log checked?	☐ Yes	☐ No ☐ N/A
Resident's known friend's apartments checked?	☐ Yes	☐ No ☐ N/A
Public bathrooms, dining room and common spaces checked?	☐ Yes	☐ No ☐ N/A
Search of community grounds initiated?	☐ Yes	☐ No ☐ N/A
Photo of resident & face sheet copied for police to take?	☐ Yes	☐ No ☐ N/A
Simulated resident located?	☐ Yes	☐ No ☐ N/A
All clear notification made to team members?	☐ Yes	☐ No ☐ N/A
Family & Police notified resident found (simulated)?	☐ Yes	☐ No ☐ N/A
For any "no" responses, describe actions taken		
Time elopement drill concluded _____ ☐ a.m. ☐ p.m.		
Critique of elopement drill		

EMPLOYEE PARTICIPATION LOG

[illegible]

6.4 Medical EmergenciesRevised 08/14/2018 Reviewed 08/14/2018

Policy Purpose

All medical situations, emergency or non-emergency are treated according to procedures set forth in accredited certification programs in Cardio Pulmonary Resuscitation (CPR) and First Aid, by persons possessing credentials for having completed those programs. Merrill Gardens care team members will possess at a minimum and in compliance with state regulation, current first aid cards and/or CPR from an accredited program.

Definitions

Major Medical Emergency

Any situation in which there is potential for loss of life, limb, or sight.

Minor Medical Emergency

Any medical situation requiring team member action such as first aid and/or calling 911.

CPR (Cardio Pulmonary Resuscitation)

A procedure which rhythmically combines chest compression with mouth-to-mouth or mouth-to-nose resuscitative breathing.

First Aid

Basic medical treatment used to stabilize injuries, stop or slow bleeding, and prevent shock.

Guidelines

Team Member Certification for Medical Care and First Aid

At a minimum, one team member per shift and in compliance with state regulation, will hold a current certification in:

- Basic level first-aid, and
- Cardiopulmonary resuscitation (CPR)

An accredited program of instruction must award cards from one of the following:

- The American Red Cross, or
- The American Heart Association, or
- State Department of Labor and Industries.

AZ: All care team members must be certified in basic Adult First-Aid and CPR. The course must include a demonstration and the card must be from The American Red Cross, The American Heart Association or The National Safety Council.

FL: Certification must be from an accredited college, university or vocational school; a licensed hospital; the American Red Cross, American Heart Association, or National Safety Council; or a provider approved by the Department of Health.

GA: Certification can be from other accredited programs.

Major Medical Emergency Procedures

A major medical emergency is one in which there is potential for loss of life, limb, or sight. All such emergency situations will be evaluated and treated by team members who possess certification in CPR and First Aid, or by licensed team, if present.

1. Stay calm.
2. In all major medical emergencies, dial 911 to summon Emergency Medical Personnel at the same time that First Aid and/or CPR are being performed. Efforts to effectively preserve life, limb or sight should be continued until the arrival of competent professional emergency medical technicians or a physician is present to assume control of the situation.

FL: The community shall have a functioning automated external defibrillator (AED) on the premises. The community is encouraged to register the location of the AED with a local emergency medical service. Staff may withhold or withdraw CPR or the use of an AED if presented with a valid order not to resuscitate.

GA: Refer to Do Not Resuscitate policy for information on whether CPR may be withheld if the resident is unresponsive.

3. If there is ever a question as to whether or not the situation is a major medical emergency, call 911. Prepare copies for paramedics or other emergency medical professionals, the following resident information:
 - Emergency contact-face sheet
 - Medical Health Statement, physician's medical evaluation, or H&P
 - Medication information, if possible
 - DNR/POLST Order, if applicable
4. Following arrival of emergency medical professionals, the designated team member in the Community will fill out an Incident Report, and, if a resident or team member, notifies their emergency point of contact. If that person is not the General Manager, then the General Manager must also be notified by telephone (or in person if in the community).
5. Notify the General Manager no matter what time of day or night. If the General Manager is unavailable, notify the Vice President of Operations.
6. If a resident is involved, the appropriate team member should notify the family or designee listed as the resident's emergency contact on the form located in the Emergency Book and in the resident's chart.
7. Notify the resident's primary physician.

NV: The resident's primary physician must be notified at the time of injury.

8. In the event the incident or accident involves potential criminal activity, then the police must also be summoned to the scene.
9. Do not, under any circumstances, make a diagnosis or in any way put yourself in danger.

GA: Any serious injury to a resident that requires medical attention must be reported to the Division of Healthcare Facility Regulation. Examples may include fractures and head injuries. Accidental or unanticipated death, actual or suspected abuse, rape, assault or battery must always be reported. The General Manager or designee shall be responsible for this report.

SC: The Community shall report every serious accident and/or incident that results in resident's death or significant loss of function or damage to a body structure, not related to the natural course of a resident's illness or underlying condition or normal course of treatment and resulting from an accident and/or incident occurring to resident within the community or on the community grounds. Serious accidents and/or incidents requiring reporting include, but are not limited to:

- Crime(s) against resident;
- Confirmed or suspected cases of abuse, neglect, or exploitation;
- Medication error with adverse reaction;
- Hospitalization as a result of the accident and/or incident;
- Severe hematoma, laceration or burn requiring medical attention or hospitalization;
- Fracture of bone or joint;
- Severe injury involving use of restraints;
- Attempted suicide; or
- Fire.

The Community shall immediately report every serious accident and/or incident to the attending physician, next-of-kin or responsible party, and the Department via telephone, email or facsimile within twenty-four (24) hours of the serious accident and/or incident. The Community shall submit a written report of its investigation of every serious accident and/or incident to the Department within five (5) days of the serious accident and/or incident. A facility's written report to the Department shall provide at a minimum:

- Facility name;
- License number;
- Type of accident and/or incident;
- Date accident and/or incident occurred;
- 5. Number of residents directly injured or affected;
- Resident record number or last four (4) digits of Social Security Number;
- Resident age and sex;
- Number of staff directly injured or affected;
- Number of visitors directly injured or affected;
- Name(s) of witness(es);
- Identified cause of accident and/or incident;
- Internal investigation results if cause unknown; and
- Brief description of the accident and/or incident including the location of occurrence and treatment of injuries.

The Community shall retain a report of every serious accident and/or incident with all of the information provided to the Department and the names, injuries, and treatments associated with each resident, staff

and/or visitor involved. The Community shall retain all serious accident and/or incident records for six (6) years after the resident stops receiving services.

“ABC” Emergency Guidelines

In medical emergencies, take the following course of action. You may find it helpful to remember the topics as a guideline acronym - ABC.

Airway

An obstructed airway is the most life-threatening of circumstances. Efforts to help an apparently unconscious and non-responsive person include:

1. Make efforts to verbally or physically rouse the person.
2. Check for presence of breathing while simultaneously checking for presence of a pulse, preferably at the neck.
3. If breathing has stopped, but a pulse is present, the airway may be blocked. Carefully remove the obstacle, if possible, that is blocking the airway.
4. If breathing and pulse have stopped, begin CPR immediately unless the resident has a valid DNR order and your state permits honoring it in that situation.

Bleeding

Profuse bleeding may be from an arterial wound. Rapid steps must be taken to slow or stop the loss of blood. Again, certified personnel should be available to assist. Steps to take include:

1. Applying direct pressure,
2. Elevating the wound(s),
3. Using tourniquets if necessary, and/or applying appropriate dressings.

NOTE: Standard Precautions should always be taken when coming into contact with bodily fluids. However, this does not preclude the application of direct pressure on a life-threatening wound, even if gloves are unavailable.

Circulation

This term refers to the presence of a heartbeat or pulse. The pulse, preferably at the neck, should always be checked upon discovering a person who is apparently unconscious and non-responsive. In the absence of a pulse, CPR must immediately begin unless the resident has a valid DNR order and your state permits honoring it in that situation.

Minor Medical Emergency Procedures

A minor medical emergency is a situation where team member action is required to protect the resident or facilitate appropriate medical attention. If in doubt, call 911.

1. Evaluate whether medical emergency is major or minor. If major, see 12.2.
2. Protect the resident from further injury or decline.
3. Provide First Aid as appropriate

4. **Notify family and physician**
5. **Assist with arranging for transportation if further medical attention is needed.**
6. **Document on Incident Report.**

6.5 Managing Aggressive and Assaultive Residents

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Disruptive behavior or abusive patterns of behavior by residents are not acceptable at Merrill Gardens. This policy defines the steps to be taken with disruptive behavior to help ensure a safe, healthy and positive environment for our residents and team members.

Definitions

Disruptive behavior or behavioral patterns

Actions initiated actively or passively by a resident which injure or threaten to injure another resident and/or team member, or themselves, or which disrupts the Community's harmonious atmosphere.

Below are examples of aggressive or assaultive behavior unacceptable at Merrill Gardens retirement communities. This behavior in all the cases listed below can be either physical or verbal in nature.

Examples are:

Aggressive

A resident might be intimidating another resident or team member through physical gestures or verbal commands or statements.

Assaultive

A resident makes an overt physical (hitting, kicking or biting) or verbal (name calling) attack on another resident or a team member. Such behavior might also be self-inflicted.

Abusive

A resident takes items from others through physical force or verbal intimidation or a resident verbally demeans others or directs coarse language at others.

Other

Either physical or verbal behavior which is disruptive and would upset the lives of the other residents. Example: A resident pounds on the wall or yells intermittently. A resident refuses to bathe, change clothes, or brush their hair.

Guidelines

Common Stressors for Residents

The following list identifies common potential causes of stress in residents' lives. By becoming familiar with these causes of stress, our team members may be more sensitive to and be able to respond more effectively to a resident's behavior.

- Loss of control over their lives and environment
- Loss of physical strength and coordination
- Loss of a sense of purpose and productivity
- Loss of independence
- Loss of memory and other thinking processes' Loss of friends and relatives through death or social isolation.

Managing an Agitated (Upset, but not Violent) Resident

When working with a resident who is agitated:

1. Give the resident something to do that is simple and within their physical ability to divert their attention, such as folding towels, straightening up the library, sweeping the front walk, etc.
2. Try to encourage an activity that will calm the resident, like listening to music, watching a movie, or playing a game.
3. Reduce noise and clutter in the resident's environment.
4. Be calm and approach the resident always in a non-threatening way, preferably from their eye level. Squat if they are sitting or sit next to them.
5. Document the behavior, interventions and response in the progress notes.
6. Notify the responsible party and the resident's medical provider and document notification.

Managing an Aggressive Resident

When dealing with an aggressive resident:

1. Respond immediately.
2. Be calm and consistent in your approach.
3. To protect the safety of other residents, remove residents from the immediate area.
4. Try to keep dangerous objects out of the resident's reach.
5. Stop whatever you are doing if you have seemingly caused the aggressiveness.
6. Assess the situation and correct any condition that may have caused the outburst.
7. Evaluate the resident for pain.
8. Use distractions to calm the resident.
9. Reduce noise immediately.
10. If resident remains aggressive or behavior escalates, call 911.

11. Use the Incident Report to document the incident. Notify the responsible party and medical provider.

Situation: Physical or Verbal Assault

If dealing with a physical or verbal assault:

1. Direct all residents to their individual apartments or out of the way of additional potential confrontation.
2. Avoid direct confrontation with the offending resident, and direct offending resident to other activities to diffuse the situation. Stay with offending resident until calm. Keep a safe distance from the resident to avoid provoking them and to provide protection. Seek assistance if needed.
3. Document incident on Incident Report. See corrective action procedure (10.7) for additional guidance.

Situation: Threat of Physical Harm

If there is a threat of physical harm:

1. If physical harm appears imminent to another resident or team member, call 911 immediately. If needed, also call crisis intervention or a mental health professional to assess the situation.
2. If needed, the law enforcement officer, along with other involved parties, will assess the situation and determine each individual's rights and responsibilities. In order for a mental health professional to detain an individual, the following conditions must exist:
 - The resident represents an immediate threat of physical danger to self or others,
 - The resident represents an immediate possibility of destroying property.
3. Resident should be evaluated by a medical provider to determine if they pose a danger to self or others.
4. Contact family or conservator/guardian regarding the incident. Enlist them to help resolve situation.
5. Contact the Vice President of Operations to determine if a move-out notice is warranted. If necessary, assist in finding appropriate housing for the resident.

Corrective Action Procedures

All such administrative actions require a witness for the Community when meeting with the resident and/or resident's conservator/guardian or responsible party. The Vice President of Operations must be informed of corrective or eviction procedures in advance.

1. After the first documented incident, meet with resident and/or resident's conservator/guardian or responsible party to review aspects of the incident. Goal: Develop mutually agreeable solutions to the problems which precipitated the incident.
2. Define the rules of behavior which must be followed in order to remain at the Community. Refer to the *Resident Handbook*.
3. Define the following possible eviction or relocation procedures if disruptive behavior persists:
 - Problem solving meeting and warning.

- Warning letter issued to resident and resident's conservator/guardian or responsible party, depending upon the situation. The letter must be approved by the Vice President of Operations.
 - Up to three (3) documented incidents within a 90-day period are grounds for a discharge notification. This notification is in the form of a letter requesting the resident to "move and vacate" the premises within thirty (30) days or less, depending upon the severity of the situation, or as regulated by state law. The move-out letter is issued to the resident and/or resident's guardian or responsible party.
 - **NOTE: An eviction or relocation must be approved by the Vice President of Operations prior to issuing the notification letter, unless there is imminent danger to others or destruction of property.**
- CA: Upon obtaining prior written approval from the licensing agency, the community may give the resident a 3 day move-out notice. Good cause exists if the resident is engaging in behavior which is a threat to the mental and/or physical health or safety of themselves or others.*
4. Prepare a written letter following the first meeting outlining the steps above and give to the resident and resident's conservator/guardian or responsible party. Place a copy in the Business Office resident's file.
 5. Document meeting discussion with the following information:
 - Date and Time
 - Location of the meeting
 - Parties present
 - Content of the discussion and copies of notices delivered to the resident and/or resident's guardian or responsible party.
 6. Keep Business Office resident's file updated with copies of all move out notices and related warnings. Complete Incident Reports as necessary. Update Service Plan as appropriate.

6.6 Death of a Resident.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

This policy seeks to provide a clear plan for handling the death of a resident. It will also assist with adhering to regulatory guidelines.

Guidelines

What To Do If A Resident Dies

In the unfortunate event of a resident's death:

1. Remain calm.
2. Secure the apartment or location of the body.
3. Unless a resident is on Hospice, or your state permits the honoring of a DNR/POLST, call 911. Refer to the Hospice plan for residents who are on Hospice services. Most of our team members are not qualified to determine if a resident is dead. If possible, have someone remain near the resident or outside the door until paramedics or the mortuary arrive. Have emergency and medical records available for review by the paramedics. Cooperate and follow instructions given by the police and paramedics.

Notifications

Make notifications as described below:

1. Notify the General Manager no matter what time of day or night.
2. The General Manager must notify the family as soon as all information is available. If the family is local, encourage them to come to the Community. If the family is away from the area, and the resident has been pronounced dead by a skilled medical professional, the General Manager may inform the family by telephone.
3. If the death is on site, the local coroner's office must be contacted to receive release of the body. The paramedics and/or police will assist.

Documentation and Resident Instructions

Complete the following as appropriate for the situation:

1. Complete a state incident report if required in your state regulations.

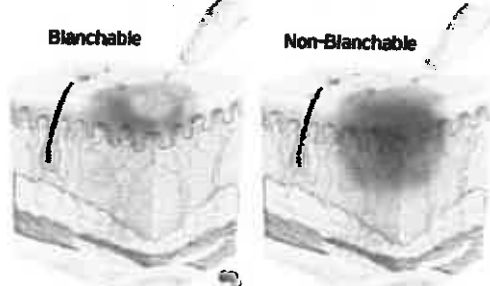
CA: Notify the LPA and Responsible party as soon as practical, but by the next working day. A Death Report (LIC624) must be submitted to the LPA within 7 days of occurrence. A copy of the Death Report is to be retained in the resident's file.

GA: The General Manager or designee shall report to the Division of Healthcare Facility Regulation whenever an accidental or unanticipated death of a resident has occurred.

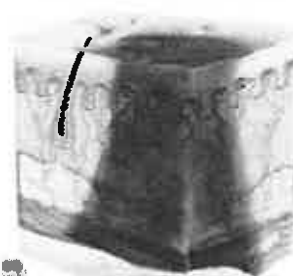
2. Each resident file may contain instructions—at the resident or responsible party's discretion—regarding what to do with the resident's body in case a family member (or responsible party) cannot be reached. The resident or responsible party has the option of providing the Community with burial instructions, and with the name of the mortuary who will make burial arrangements.
3. If a resident dies without known next of kin or guardian, immediately contact your county public administrator or the appointed public guardian.
4. Once the resident's body is removed from the community, ensure that the apartment is locked.
5. Complete the *Incident Report*.

Pressure Injury Staging-2016 NPUAP (National Pressure Ulcer Advisory Panel)

Blanchable vs Non-Blanchable



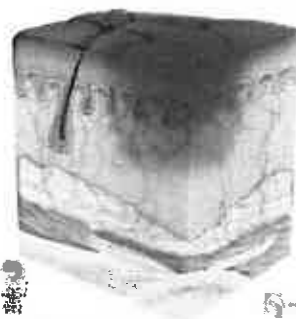
Deep Tissue Pressure Injury



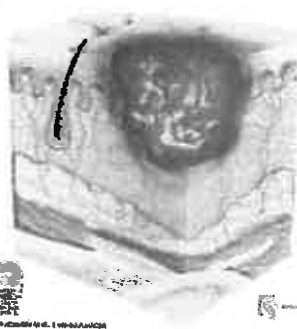
Stage 1 Pressure Injury - Caucasian



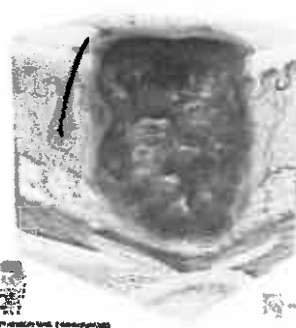
Stage 1 Pressure Injury - Non-Caucasian



Stage 2 Pressure Injury



Stage 3 Pressure Injury



Stage 4 Pressure Injury



Unstageable Pressure Injury - Slough and Eschar



